

291.2 *Other alcoholic dementia*

Nonhallucinatory dementias occurring in association with alcoholism but not characterized by the features of either delirium tremens or Korsakov's psychosis.

Alcoholic dementia NOS

Chronic alcoholic brain syndrome

291.3 *Other alcoholic hallucinosis*

A psychosis usually of less than six months' duration, with slight or no clouding of consciousness and much anxious restlessness in which auditory hallucinations, mostly of voices uttering insults and threats, predominate.

Excludes: schizophrenia (295.—) and paranoid states (297.—) taking the form of chronic hallucinosis with clear consciousness in an alcoholic

291.4 *Pathological drunkenness*

Acute psychotic episodes induced by relatively small amounts of alcohol. These are regarded as individual idiosyncratic reactions to alcohol, not due to excessive consumption and without conspicuous neurological signs of intoxication.

Excludes: simple drunkenness (305.0)

291.5 *Alcoholic jealousy*

Chronic paranoid psychosis characterized by delusional jealousy and associated with alcoholism.

Alcoholic paranoia

Excludes: nonalcoholic paranoid states (297.—)
schizophrenia, paranoid type (295.3)

291.8 *Other*

Alcoholic withdrawal syndrome

Excludes: delirium tremens (291.0)

291.9 *Unspecified*

Alcoholic:

mania NOS

psychosis NOS

Alcoholism (chronic) with psychosis

292 **Drug psychoses**

Syndromes that do not fit the descriptions given in 295-298 (nonorganic psychoses) and which are due to consumption of drugs [notably amphetamines, barbiturates and the opiate and LSD groups] and solvents. Some of the syndromes in this group are not as severe as most conditions labelled "psychotic" but they are included here for practical reasons. Use additional E Code to identify the drug and also code drug dependence (304.—) if present.

292.0 Drug withdrawal syndrome

States associated with drug withdrawal ranging from severe, as specified for alcohol under 291.0 (delirium tremens) to less severe characterized by one or more symptoms such as convulsions, tremor, anxiety, restlessness, gastrointestinal and muscular complaints, and mild disorientation and memory disturbance.

292.1 Paranoid and/or hallucinatory states induced by drugs

States of more than a few days but not usually of more than a few months duration, associated with large or prolonged intake of drugs, notably of the amphetamine and LSD groups. Auditory hallucinations usually predominate, and there may be anxiety and restlessness.

Excludes: the described conditions with confusion or delirium (293.—)
states following LSD or other hallucinogens, lasting only a few days or less ["bad trips"] (305.3)

292.2 Pathological drug intoxication

Individual idiosyncratic reactions to comparatively small quantities of a drug, which take the form of acute, brief psychotic states of any type.

Excludes: physiological side-effects of drugs [e.g., dystonias]
expected brief psychotic reactions to hallucinogens ["bad trips"] (305.3)

292.8 Other**292.9 Unspecified****293 Transient organic psychotic conditions**

States characterized by clouded consciousness, confusion, disorientation, illusions and often vivid hallucinations. They are usually due to some intra- or extracerebral toxic, infectious, metabolic or other systemic disturbance and are generally reversible. Depressive and paranoid symptoms may also be present but are not the main feature. Use additional code to identify the associated physical or neurological condition.

Excludes: confusional state or delirium superimposed on senile dementia (290.3)
dementia due to:
alcohol (291.—)
arteriosclerosis (290.4)
senility (290.0)

293.0 Acute confusional state

Short-lived states, lasting hours or days, of the above type.

Acute:

delirium
infective psychosis
organic reaction
post-traumatic organic
psychosis

Acute:

psycho-organic syndrome
psychosis associated with endocrine, metabolic or cerebrovascular disorder

Epileptic:

confusional state
twilight state

293.1 Subacute confusional state

States of the above type in which the symptoms, usually less florid, last for several weeks or longer, during which they may show marked fluctuations in intensity.

Subacute:

delirium
infective psychosis
organic reaction
post-traumatic organic psychosis

Subacute:

psycho-organic syndrome
psychosis associated with endo-
crine or metabolic disorder

293.8 Other

293.9 Unspecified

294 Other organic psychotic conditions (chronic)

294.0 Korsakov's psychosis or syndrome (nonalcoholic)

Syndromes as described under 291.1 but not due to alcohol.

294.1 Dementia in conditions classified elsewhere

Dementia not classifiable as senile, presenile or arteriosclerotic (290.-) but associated with other underlying conditions.

Dementia in:

cerebral lipidoses
epilepsy
general paralysis of the insane
hepatolenticular degeneration
Huntington's chorea
multiple sclerosis
polyarteritis nodosa

Use additional code to identify the underlying physical condition

294.8 Other

States that fulfill the criteria of an organic psychosis but do not take the form of a confusional state (293.-), a nonalcoholic Korsakov's psychosis (294.0) or a dementia (294.1).

Mixed paranoid and affective
organic psychotic states

Epileptic psychosis NOS (code
also 345.-)

Excludes: mild memory disturbances, not amounting to dementia (310.1)

294.9 Unspecified

OTHER PSYCHOSES (295-299)

295 Schizophrenic psychoses

A group of psychoses in which there is a fundamental disturbance of personality, a characteristic distortion of thinking, often a sense of being controlled by alien forces, delusions which may be bizarre, disturbed perception, abnormal affect out of keeping with the real

situation, and autism. Nevertheless, clear consciousness and intellectual capacity are usually maintained. The disturbance of personality involves its most basic functions which give the normal person his feeling of individuality, uniqueness and self-direction. The most intimate thoughts, feelings and acts are often felt to be known to or shared by others and explanatory delusions may develop, to the effect that natural or supernatural forces are at work to influence the schizophrenic person's thoughts and actions in ways that are often bizarre. He may see himself as the pivot of all that happens. Hallucinations, especially of hearing, are common and may comment on the patient or address him. Perception is frequently disturbed in other ways; there may be perplexity, irrelevant features may become all-important and, accompanied by passivity feelings, may lead the patient to believe that everyday objects and situations possess a special, usually sinister, meaning intended for him. In the characteristic schizophrenic disturbance of thinking, peripheral and irrelevant features of a total concept, which are inhibited in normal directed mental activity, are brought to the forefront and utilized in place of the elements relevant and appropriate to the situation. Thus thinking becomes vague, elliptical and obscure, and its expression in speech sometimes incomprehensible. Breaks and interpolations in the flow of consecutive thought are frequent, and the patient may be convinced that his thoughts are being withdrawn by some outside agency. Mood may be shallow, capricious or incongruous. Ambivalence and disturbance of volition may appear as inertia, negativism or stupor. Catatonia may be present. The diagnosis "schizophrenia" should not be made unless there is, or has been evident during the same illness, characteristic disturbance of thought, perception, mood, conduct, or personality—preferably in at least two of these areas. The diagnosis should not be restricted to conditions running a protracted, deteriorating, or chronic course. In addition to making the diagnosis on the criteria just given, effort should be made to specify one of the following subdivisions of schizophrenia, according to the predominant symptoms.

Includes: schizophrenia of the types described in 295.0-295.9 occurring in children

Excludes: childhood type schizophrenia (299.9)
infantile autism (299.0)

295.0 *Simple type*

A psychosis in which there is insidious development of oddities of conduct, inability to meet the demands of society, and decline in total performance. Delusions and hallucinations are not in evidence and the condition is less obviously psychotic than are the hebephrenic, catatonic and paranoid types of schizophrenia. With increasing social impoverishment vagrancy may ensue and the patient becomes self-absorbed, idle and aimless. Because the schizophrenic symptoms are not clear-cut, diagnosis of this form should be made sparingly, if at all.

Schizophrenia simplex

Excludes: latent schizophrenia (295.5)

295.1 *Hebephrenic type*

A form of schizophrenia in which affective changes are prominent, delusions and hallucinations fleeting and fragmentary, behaviour irresponsible and unpredictable and mannerisms common. The mood is shallow and inappropriate, accompanied by giggling or self-satisfied, self-absorbed smiling, or by a lofty manner, grimaces, mannerisms, pranks, hypochondriacal complaints and reiterated phrases. Thought is disorganized. There is a tendency to remain solitary, and behaviour seems empty of purpose and feeling. This form of schizophrenia usually starts between the ages of 15 and 25 years.

Hebephrenia

295.2 *Catatonic type*

Includes as an essential feature prominent psychomotor disturbances often alternating between extremes such as hyperkinesia and stupor, or automatic obedience and negativism. Constrained attitudes may be maintained for long periods: if the patient's limbs are put in some unnatural position they may be held there for some time after the external force has been removed. Severe excitement may be a striking feature of the condition. Depressive or hypomanic concomitants may be present.

Catatonic:	Schizophrenic:
agitation	catalepsy
excitation	catatonia
stupor	flexibilitas cerea

295.3 *Paranoid type*

The form of schizophrenia in which relatively stable delusions, which may be accompanied by hallucinations, dominate the clinical picture. The delusions are frequently of persecution but may take other forms [for example of jealousy, exalted birth, Messianic mission, or bodily change]. Hallucinations and erratic behaviour may occur; in some cases conduct is seriously disturbed from the outset, thought disorder may be gross, and affective flattening with fragmentary delusions and hallucinations may develop.

Paraphrenic schizophrenia

Excludes: paraphrenia, involutional paranoid state (297.2)
paranoia (297.1)

295.4 *Acute schizophrenic episode*

Schizophrenic disorders, other than those listed above, in which there is a dream-like state with slight clouding of consciousness and perplexity. External things, people and events may become charged with personal significance for the patient. There may be ideas of reference and emotional turmoil. In many such cases remission occurs within a few weeks or months, even without treatment.

Oneirophrenia	Schizophreniform:
	attack
	psychosis, confusional type

Excludes: acute forms of schizophrenia of:
catatonic type (295.2)
hebephrenic type (295.1)
paranoid type (295.3)
simple type (295.0)

295.5 *Latent schizophrenia*

It has not been possible to produce a generally acceptable description for this condition. It is not recommended for general use, but a description is provided for those who believe it to be useful: a condition of eccentric or inconsequent behaviour and anomalies of affect which give the impression of schizophrenia though no definite and characteristic schizophrenic anomalies, present or past, have been manifest.

The inclusion terms indicate that this is the best place to classify some other poorly defined varieties of schizophrenia.

Latent schizophrenic reaction	Schizophrenia:
Schizophrenia:	pseudoneurotic
borderline	pseudopsychopathic
prepsychotic	
prodromal	

Excludes: schizoid personality (301.2)

295.6 *Residual schizophrenia*

A chronic form of schizophrenia in which the symptoms that persist from the acute phase have mostly lost their sharpness. Emotional response is blunted and thought disorder, even when gross, does not prevent the accomplishment of routine work.

Chronic undifferentiated schizophrenia
 Restzustand (schizophrenic)
 Schizophrenic residual state

295.7 *Schizoaffective type*

A psychosis in which pronounced manic or depressive features are intermingled with schizophrenic features and which tends towards remission without permanent defect, but which is prone to recur. The diagnosis should be made only when both the affective and schizophrenic symptoms are pronounced.

Cyclic schizophrenia
 Mixed schizophrenic and affective psychosis
 Schizoaffective psychosis
 Schizophreniform psychosis, affective type

295.8 *Other*

Schizophrenia of specified type not classifiable under 295.0-295.7.

Acute (undifferentiated)	Atypical schizophrenia
schizophrenia	Coenesthopathic schizophrenia

Excludes: infantile autism (299.0)

295.9 *Unspecified*

To be used only as a last resort.

Schizophrenia NOS
 Schizophrenic reaction NOS
 Schizophreniform psychosis NOS

296 *Affective psychoses*

Mental disorders, usually recurrent, in which there is a severe disturbance of mood [mostly compounded of depression and anxiety but also manifested as elation and excitement] which is accompanied by one or more of the following: delusions, perplexity, disturbed attitude to

self, disorder of perception and behaviour; these are all in keeping with the patient's prevailing mood [as are hallucinations when they occur]. There is a strong tendency to suicide. For practical reasons, mild disorders of mood may also be included here if the symptoms match closely the descriptions given; this applies particularly to mild hypomania.

Excludes: reactive depressive psychosis (298.0)
reactive excitation (298.1)
neurotic depression (300.4)

296.0 *Manic-depressive psychosis, manic type*

Mental disorders characterized by states of elation or excitement out of keeping with the patient's circumstances and varying from enhanced liveliness [hypomania] to violent, almost uncontrollable excitement. Aggression and anger, flight of ideas, distractibility, impaired judgement, and grandiose ideas are common.

Hypomania NOS	Manic psychosis
Hypomanic psychosis	Manic-depressive psychosis or
Mania (monopolar) NOS	reaction:
Manic disorder	hypomanic
	manic

Excludes: circular type if there was a previous attack of depression (296.2)

296.1 *Manic-depressive psychosis, depressed type*

An affective psychosis in which there is a widespread depressed mood of gloom and wretchedness with some degree of anxiety. There is often reduced activity but there may be restlessness and agitation. There is a marked tendency to recurrence; in a few cases this may be at regular intervals.

Depressive psychosis	Manic-depressive reaction,
Endogenous depression	depressed
Involutional melancholia	Monopolar depression
	Psychotic depression

Excludes: circular type if previous attack was of manic type (296.3)
depression NOS (311)

296.2 *Manic-depressive psychosis, circular type but currently manic*

An affective psychosis which has appeared in both the depressive and the manic form, either alternating or separated by an interval of normality, but in which the manic form is currently present. [The manic phase is far less frequent than the depressive].

Bipolar disorder, now manic

Excludes: brief compensatory or rebound mood swings (296.8)

296.3 *Manic-depressive psychosis, circular type but currently depressed*

Circular type (see 296.2) in which the depressive form is currently present.

Bipolar disorder, now depressed

Excludes: brief compensatory or rebound mood swings (296.8)

296.4 *Manic-depressive psychosis, circular type, mixed*

An affective psychosis in which both manic and depressive symptoms are present at the same time.

296.5 *Manic-depressive psychosis, circular type, current condition not specified*

Circular type (see 296.2) in which the current condition is not specified as either manic or depressive.

296.6 *Manic-depressive psychosis, other and unspecified*

Use this code for cases where no other information is available, except the unspecified term, manic-depressive psychosis, or for syndromes corresponding to the descriptions of depressed (296.1) or manic (296.0) types but which for other reasons cannot be classified under 296.0-296.5.

Manic-depressive psychosis:

NOS

mixed type

Manic-depressive:

reaction NOS

syndrome NOS

296.8 *Other*

Excludes: psychogenic affective psychoses (298.-)

296.9 *Unspecified*

Affective psychosis NOS

Melancholia NOS

297 *Paranoid states*

Excludes: acute paranoid reaction (298.3)

alcoholic jealousy (291.5)

paranoid schizophrenia (295.3)

297.0 *Paranoid state, simple*

A psychosis, acute or chronic, not classifiable as schizophrenia or affective psychosis, in which delusions, especially of being influenced, persecuted or treated in some special way, are the main symptoms. The delusions are of a fairly fixed, elaborate and systematized kind.

297.1 *Paranoia*

A rare chronic psychosis in which logically constructed systematized delusions have developed gradually without concomitant hallucinations or the schizophrenic type of disordered thinking. The delusions are mostly of grandeur [the paranoiac prophet or inventor], persecution or somatic abnormality.

Excludes: paranoid personality disorder (301.0)

297.2 *Paraphrenia*

Paranoid psychosis in which there are conspicuous hallucinations, often in several modalities. Affective symptoms and disordered thinking, if present, do not dominate the clinical picture and the personality is well preserved.

Involutional paranoid state
Late paraphrenia

297.3 Induced psychosis

Mainly delusional psychosis, usually chronic and often without florid features, which appears to have developed as a result of a close, if not dependent, relationship with another person who already has an established similar psychosis. The delusions are at least partly shared. The rare cases in which several persons are affected should also be included here.

Folie à deux	Induced paranoid disorder
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297.8 Other

Paranoid states which, though in many ways akin to schizophrenic or affective states, cannot readily be classified under any of the preceding rubrics, nor under 298.4.

Paranoia querulans	Sensitiver Beziehungswahn
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Excludes: senile paranoid state (297.2)

297.9 *Unspecified*

Paranoid:
psychosis NOS
reaction NOS
state NOS

298 Other nonorganic psychoses

Categories 298.0-298.8 should be restricted to the small group of psychotic conditions that are largely or entirely attributable to a recent life experience. They should not be used for the wider range of psychoses in which environmental factors play some [but not the *major*] part in aetiology.

298.0 *Depressive type*

A depressive psychosis which can be similar in its symptoms to manic-depressive psychosis, depressed type (296.1) but is apparently provoked by saddening stress such as a bereavement, or a severe disappointment or frustration. There may be less diurnal variation of symptoms than in 296.1, and the delusions are more often understandable in the context of the life experiences. There is usually a serious disturbance of behaviour, e.g., major suicidal attempt.

Reactive depressive psychosis
Psychogenic depressive psychosis

Excludes: manic-depressive psychosis, depressed type (296.1)
neurotic depression (300.4)

298.1 *Excitative type*

An affective psychosis similar in its symptoms to manic-depressive psychosis, manic type, but apparently provoked by emotional stress.

Excludes: manic-depressive psychosis, manic type (296.0)

298.2 *Reactive confusion*

Mental disorders with clouded consciousness, disorientation [though less marked than in organic confusion] and diminished accessibility often accompanied by excessive activity and apparently provoked by emotional stress.

Psychogenic confusion

Psychogenic twilight state

Excludes: acute confusional state (293.0)

298.3 *Acute paranoid reaction*

Paranoid states apparently provoked by some emotional stress. The stress is often misconstrued as an attack or threat. Such states are particularly prone to occur in prisoners or as acute reactions to a strange and threatening environment, e.g., in immigrants.

Bouffée délirante

Excludes: paranoid states (297.-)

298.4 *Psychogenic paranoid psychosis*

Psychogenic or reactive paranoid psychosis of any type which is more protracted than the acute reactions covered in 298.3. Where there is a diagnosis of psychogenic paranoid psychosis which does not specify "acute" this coding should be made.

Protracted reactive paranoid psychosis

298.8 *Other and unspecified reactive psychosis*

Hysterical psychosis

Psychogenic stupor

Psychogenic psychosis NOS

298.9 *Unspecified psychosis*

To be used only as a last resort, when no other term can be used.

Psychosis NOS

299 *Psychoses with origin specific to childhood*

This category should be used only for psychoses which always begin before puberty. Adult-type psychoses such as schizophrenia or manic-depressive psychoses when occurring in childhood should be coded elsewhere under the appropriate heading—i.e., 295 and 296 for the examples given.

299.0 *Infantile autism*

A syndrome present from birth or beginning almost invariably in the first 30 months. Responses to auditory and sometimes to visual stimuli are abnormal and there are usually severe problems in the understanding of spoken language. Speech is delayed and, if it develops, is characterized by echolalia, the reversal of pronouns, immature grammatical structure and inability to use abstract terms. There is generally an impairment in the social

Excludes: schizophrenia of adult type occurring in childhood (295.0-295.8)

NEUROTIC DISORDERS, PERSONALITY DISORDERS AND OTHER
NONPSYCHOTIC MENTAL DISORDERS (300-316)

300 *Neurotic disorders*

The distinction between neurosis and psychosis is difficult and remains subject to debate. However, it has been retained in view of its wide use.

Neurotic disorders are mental disorders without any demonstrable organic basis in which the patient may have considerable insight and has unimpaired reality testing, in that he usually does not confuse his morbid subjective experiences and fantasies with external reality. Behaviour may be greatly affected although usually remaining within socially acceptable limits, but personality is not disorganized. The principal manifestations include excessive anxiety, hysterical symptoms, phobias, obsessional and compulsive symptoms, and depression.

300.0 *Anxiety states*

Various combinations of physical and mental manifestations of anxiety, not attributable to real danger and occurring either in attacks or as a persisting state. The anxiety is usually diffuse and may extend to panic. Other neurotic features such as obsessional or hysterical symptoms may be present but do not dominate the clinical picture.

Anxiety:	Panic:
neurosis	attack
reaction	disorder
state (neurotic)	state

Excludes: neurasthenia (300.5)
psychophysiological disorders (306.-)

300.1 *Hysteria*

Mental disorders in which motives, of which the patient seems unaware, produce either a restriction of the field of consciousness or disturbances of motor or sensory function which may seem to have psychological advantage or symbolic value. It may be characterized by conversion phenomena or dissociative phenomena. In the conversion form the chief or only symptoms consist of psychogenic disturbance of function in some part of the body, e.g., paralysis, tremor, blindness, deafness, seizures. In the dissociative variety, the most prominent feature is a narrowing of the field of consciousness which seems to serve an unconscious purpose and is commonly accompanied or followed by a selective amnesia. There may be dramatic but essentially superficial changes of personality sometimes taking the form of a fugue [wandering state]. Behaviour may mimic psychosis or, rather, the patient's idea of psychosis.

Astasia-abasia, hysterical	Dissociative reaction or state
Compensation neurosis	Ganser's syndrome, hysterical
Conversion hysteria	Hysteria NOS
Conversion reaction	Multiple personality

Excludes: adjustment reaction (309.-)
anorexia nervosa (307.1)
gross stress reaction (308.-)
hysterical personality (301.5)
psychophysiological disorders (306.-)

300.2 *Phobic state*

Neurotic states with abnormally intense dread of certain objects or specific situations which would not normally have that effect. If the anxiety tends to spread from a specified situation or object to a wider range of circumstances, it becomes akin to or identical with anxiety state, and should be classified as such (300.0).

Agoraphobia	Claustrophobia
Animal phobias	Phobia NOS
Anxiety-hysteria	

Excludes: anxiety state (300.0)
obsessional phobias (300.3)

300.3 *Obsessive-compulsive disorders*

States in which the outstanding symptom is a feeling of subjective compulsion—which must be resisted—to carry out some action, to dwell on an idea, to recall an experience, or to ruminate on an abstract topic. Unwanted thoughts which intrude, the insistency of words or ideas, ruminations or trains of thought are perceived by the patient to be inappropriate or nonsensical. The obsessional urge or idea is recognized as alien to the personality but as coming from within the self. Obsessional actions may be quasi-ritual performances designed to relieve anxiety e.g., washing the hands to cope with contamination. Attempts to dispel the unwelcome thoughts or urges may lead to a severe struggle, with intense anxiety.

Anankastic neurosis
Compulsive neurosis

Excludes: obsessive-compulsive symptoms occurring in:
endogenous depression (296.1)
schizophrenia (295.-)
organic states, e.g., encephalitis

300.4 *Neurotic depression*

A neurotic disorder characterized by disproportionate depression which has usually recognizably ensued on a distressing experience; it does not include among its features delusions or hallucinations, and there is often preoccupation with the psychic trauma which preceded the illness, e.g., loss of a cherished person or possession. Anxiety is also frequently present and mixed states of anxiety and depression should be included here. The distinction between depressive neurosis and psychosis should be made not only upon the degree of depression but also on the presence or absence of other neurotic and psychotic characteristics and upon the degree of disturbance of the patient's behaviour.

Anxiety depression	Neurotic depressive state
Depressive reaction	Reactive depression

Excludes: adjustment reaction with depressive symptoms (309.0)
depression NOS (311)
manic-depressive psychosis, depressed type (296.1)
reactive depressive psychosis (298.0)

300.5 *Neurasthenia*

A neurotic disorder characterized by fatigue, irritability, headache, depression, insomnia, difficulty in concentration, and lack of capacity for enjoyment [anhedonia]. It may follow

or accompany an infection or exhaustion, or arise from continued emotional stress. If neurasthenia is associated with a physical disorder, the latter should also be coded.

Nervous debility

Excludes: anxiety state (300.0)
 neurotic depression (300.4)
 psychophysiological disorders (306.-)
 specific nonpsychotic mental disorders following organic
 brain damage (310.-)

300.6 *Depersonalization syndrome*

A neurotic disorder with an unpleasant state of disturbed perception in which external objects or parts of one's own body are experienced as changed in their quality, unreal, remote or automatized. The patient is aware of the subjective nature of the change he experiences. Depersonalization may occur as a feature of several mental disorders including depression, obsessional neurosis, anxiety and schizophrenia; in that case the condition should not be classified here but in the corresponding major category.

Derealization (neurotic)

300.7 *Hypochondriasis*

A neurotic disorder in which the conspicuous features are excessive concern with one's health in general or the integrity and functioning of some part of one's body, or, less frequently, one's mind. It is usually associated with anxiety and depression. It may occur as a feature of severe mental disorder and in that case should not be classified here but in the corresponding major category.

Excludes: hysteria (300.1)
 manic-depressive psychosis, depressed type (296.1)
 neurasthenia (300.5)
 obsessional disorder (300.3)
 schizophrenia (295.-)

300.8 *Other neurotic disorders*

Neurotic disorders not classified elsewhere, e.g., occupational neurosis. Patients with mixed neuroses should not be classified in this category but according to the most prominent symptoms they display.

Briquet's disorder

Occupational neurosis, including writer's cramp

Psychasthenia

Psychasthenic neurosis

300.9 *Unspecified*

To be used only as a last resort.

Neurosis NOS

Psychoneurosis NOS

301 **Personality disorders**

Deeply ingrained maladaptive patterns of behaviour generally recognizable by the time of adolescence or earlier and continuing throughout most of adult life, although often

becoming less obvious in middle or old age. The personality is abnormal either in the balance of its components, their quality and expression or in its total aspect. Because of this deviation or psychopathy the patient suffers or others have to suffer and there is an adverse effect upon the individual or on society. It includes what is sometimes called psychopathic personality, but if this is determined primarily by malfunctioning of the brain, it should not be classified here but as one of the nonpsychotic organic brain syndromes (310). When the patient exhibits an anomaly of personality directly related to his neurosis or psychosis, e.g., schizoid personality and schizophrenia or anankastic personality and obsessive compulsive neurosis, the relevant neurosis or psychosis which is in evidence should be diagnosed in addition.

Character neurosis

301.0 *Paranoid personality disorder*

Personality disorder in which there is excessive sensitiveness to setbacks or to what are taken to be humiliations and rebuffs, a tendency to distort experience by misconstruing the neutral or friendly actions of others as hostile or contemptuous, and a combative and tenacious sense of personal rights. There may be a proneness to jealousy or excessive self-importance. Such persons may feel helplessly humiliated and put upon; others, likewise excessively sensitive, are aggressive and insistent. In all cases there is excessive self-reference.

Fanatic personality

Paranoid personality (disorder)

Paranoid traits

Excludes: acute paranoid reaction (298.3)
alcoholic paranoia (291.5)
paranoid schizophrenia (295.3)
paranoid states (297.-)

301.1 *Affective personality disorder*

Personality disorder characterized by lifelong predominance of a pronounced mood which may be persistently depressive, persistently elated, or alternately one then the other. During periods of elation there is unshakeable optimism and an enhanced zest for life and activity, whereas periods of depression are marked by worry, pessimism, low output of energy and a sense of futility.

Cycloid personality

Depressive personality

Cyclothymic personality

Excludes: affective psychoses (296.-)
cyclothymia (296.2-296.5)
neurasthenia (300.5)
neurotic depression (300.4)

301.2 *Schizoid personality disorder*

Personality disorder in which there is withdrawal from affectional, social and other contacts with autistic preference for fantasy and introspective reserve. Behaviour may be slightly eccentric or indicate avoidance of competitive situations. Apparent coolness and detachment may mask an incapacity to express feeling.

Excludes: schizophrenia (295.-)

301.3 *Explosive personality disorder*

Personality disorder characterized by instability of mood with liability to intemperate outbursts of anger, hate, violence or affection. Aggression may be expressed in words or in physical violence. The outbursts cannot readily be controlled by the affected persons, who are not otherwise prone to antisocial behaviour.

Aggressive: personality reaction	Emotional instability (excessive) Pathological emotionality Quarrelsomeness
Aggressiveness	

Excludes: dyssocial personality (301.7)
hysterical neurosis (300.1)

301.4 *Anankastic personality disorder*

Personality disorder characterized by feelings of personal insecurity, doubt and incompleteness leading to excessive conscientiousness, checking, stubbornness and caution. There may be insistent and unwelcome thoughts or impulses which do not attain the severity of an obsessional neurosis. There is perfectionism and meticulous accuracy and a need to check repeatedly in an attempt to ensure this. Rigidity and excessive doubt may be conspicuous.

Compulsive personality	Obsessional personality
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Excludes: obsessive-compulsive disorder (300.3)
phobic state (300.2)

301.5 *Hysterical personality disorder*

Personality disorder characterized by shallow, labile affectivity, dependence on others, craving for appreciation and attention, suggestibility and theatricality. There is often sexual immaturity, e.g., frigidity and over-responsiveness to stimuli. Under stress hysterical symptoms [neurosis] may develop.

Histrionic personality	Psychoinfantile personality
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Excludes: hysterical neurosis (300.1)

301.6 *Asthenic personality disorder*

Personality disorder characterized by passive compliance with the wishes of elders and others and a weak inadequate response to the demands of daily life. Lack of vigour may show itself in the intellectual or emotional spheres; there is little capacity for enjoyment.

Dependent personality	Passive personality
Inadequate personality	

Excludes: neurasthenia (300.5)

301.7 *Personality disorder with predominantly sociopathic or asocial manifestation*

Personality disorder characterized by disregard for social obligations, lack of feeling for others, and impetuous violence or callous unconcern. There is a gross disparity between behaviour and the prevailing social norms. Behaviour is not readily modifiable by experi-

ence, including punishment. People with this personality are often affectively cold and may be abnormally aggressive or irresponsible. Their tolerance to frustration is low; they blame others or offer plausible rationalizations for the behaviour which brings them into conflict with society.

Amoral personality

Asocial personality

Antisocial personality

Excludes: disturbance of conduct without specifiable personality disorder

(312.-)

explosive personality (301.3)

301.8 *Other personality disorders*

Personality:

eccentric

"haltlose" type

Personality:

immature

passive-aggressive

psychoneurotic

Excludes: psychoinfantile personality (301.5)

301.9 *Unspecified*

Pathological personality NOS

Personality disorder NOS

Psychopathic:

constitutional state

personality (disorder)

302 *Sexual deviations and disorders*

Abnormal sexual inclinations or behaviour which are part of a referral problem. The limits and features of normal sexual inclination and behaviour have not been stated absolutely in different societies and cultures but are broadly such as serve approved social and biological purposes. The sexual activity of affected persons is directed primarily either towards people not of the opposite sex, or towards sexual acts not associated with coitus normally or towards coitus performed under abnormal circumstances. If the anomalous behaviour becomes manifest only during psychosis or other mental illness the condition should be classified under the major illness. It is common for more than one anomaly to occur together in the same individual; in the case the predominant deviation is classified. It is preferable not to include in this category individuals who perform deviant sexual acts when normal sexual outlets are not available to them.

302.0 *Homosexuality*

Exclusive or predominant sexual attraction for persons of the same sex with or without physical relationship. Code homosexuality here whether or not it is considered as a mental disorder.

Lesbianism

Excludes: homosexual paedophilia (302.2)

302.1 *Bestiality*

Sexual or anal intercourse with animals.

302.2 *Paedophilia*

Sexual deviations in which an adult engages in sexual activity with a child of the same or opposite sex.

302.3 *Transvestism*

Sexual deviation in which sexual pleasure is derived from dressing in clothes of the opposite sex. There is no consistent attempt to take on the identity or behaviour of the opposite sex.

Excludes: trans-sexualism (302.5)

302.4 *Exhibitionism*

Sexual deviation in which the main sexual pleasure and gratification is derived from exposure of the genitals to a person of the opposite sex.

302.5 *Trans-sexualism*

Sexual deviation centered around fixed beliefs that the overt bodily sex is wrong. The resulting behaviour is directed towards either changing the sexual organs by operation, or completely concealing the bodily sex by adopting both the dress and behaviour of the opposite sex.

Excludes: transvestism (302.3)

302.6 *Disorders of psychosexual identity*

Behaviour occurring in preadolescents of immature psychosexuality which is similar to that shown in the sexual deviations described under transvestism (302.3) and trans-sexualism (302.5). Cross-dressing is intermittent, although it may be frequent, and identification with the behaviour and appearance of the opposite sex is not yet fixed. The commonest form is feminism in boys.

Gender-role disorder

Excludes: homosexuality (302.0)
trans-sexualism (302.5)
transvestism (302.3)

302.7 *Frigidity and impotence*

Frigidity—dislike of or aversion to sexual intercourse, of psychological origin, of sufficient intensity to lead, if not to active avoidance, to marked anxiety, discomfort or pain when normal sexual intercourse takes place. Less severe degrees of this disorder that also give rise to consultation should also be coded here.

Impotence—sustained inability, due to psychological causes, to maintain an erection which will allow normal heterosexual penetration and ejaculation to take place.

Dyspareunia, psychogenic

Excludes: impotence of organic origin
normal transient symptoms from ruptured hymen
transient or occasional failures of erection due to fatigue,
anxiety, alcohol or drugs

302.8 *Other*

Fetishism

Sadism

Masochism

302.9 *Unspecified*

303 Alcohol dependence syndrome

A state, psychic and usually also physical, resulting from taking alcohol, characterized by behavioural and other responses that always include a compulsion to take alcohol on a continuous or periodic basis in order to experience its psychic effects, and sometimes to avoid the discomfort of its absence; tolerance may or may not be present. A person may be dependent on alcohol and other drugs; if so also make the appropriate 304 coding. If dependence is associated with alcoholic psychosis or with physical complications, *both* should be coded.

Acute drunkenness in alcoholism Dipsomania

Chronic alcoholism

Excludes: alcoholic psychoses (291.-)
 drunkenness NOS (305.0)
 physical complications of alcohol, such as:
 cirrhosis of liver (571.2)
 epilepsy (345.-)
 gastritis (535.3)

304 Drug dependence

A state, psychic and sometimes also physical, resulting from taking a drug, characterized by behavioural and other responses that always include a compulsion to take a drug on a continuous or periodic basis in order to experience its psychic effects, and sometimes to avoid the discomfort of its absence. Tolerance may or may not be present. A person may be dependent on more than one drug.

Excludes: nondependent abuse of drugs (305.-)

304.0 *Morphine type*

Heroin	Opium alkaloids and their derivatives
Methadone	Synthetics with morphine-like effects
Opium	

304.1 *Barbiturate type*

Barbiturates
 Nonbarbiturate sedatives and tranquillizers with a similar effect:
 chlordiazepoxide
 diazepam
 glutethimide
 meprobamate

304.2 *Cocaine*

Coca leaves and derivatives

304.3 *Cannabis*

Hemp	Marijuana
Hashish	

Tobacco dependence

305.2 *Cannabis*

305.3 *Hallucinogens*

Cases of acute intoxication or "bad trips".

LSD reaction

305.4 *Barbiturates and tranquilizers*

Cases where a person has taken the drug to the detriment of his health or social functioning, in doses above or for periods beyond those normally regarded as therapeutic.

305.5 *Morphine type*

305.6 *Cocaine type*

305.7 *Amphetamine type*

305.8 *Antidepressants*

305.9 *Other, mixed or unspecified*

"Laxative habit"

Misuse of drugs NOS

Nonprescribed use of drugs or
patent medicinals

306 Physiological malfunction arising from mental factors

A variety of physical symptoms or types of physiological malfunction of mental origin, not involving tissue damage and usually mediated through the autonomic nervous system. The disorders are grouped according to body system. Codes 306.0-306.9 should not be used if the physical symptom is secondary to a psychiatric disorder classifiable elsewhere. If tissue damage is involved, code under 316.

Excludes: hysteria (300.1)

psychic factors associated with physical conditions involving
tissue damage classified elsewhere (316)

specific nonpsychotic mental disorders following organic
brain damage (310.-)

306.0 *Musculoskeletal*

Psychogenic torticollis

Excludes: Gilles de la Tourette's syndrome (307.2)
tics (307.2)

306.1 *Respiratory*

Air hunger

Hiccough (psychogenic)

Hyperventilation

Psychogenic cough

Yawning

Excludes: psychogenic asthma (316 and 493.9)

306.2 *Cardiovascular*

Cardiac neurosis	Neurocirculatory asthenia
Cardiovascular neurosis	Psychogenic cardiovascular disorder

Excludes: psychogenic paroxysmal tachycardia (316 and 427.9)

306.3 *Skin*

Psychogenic pruritus

Excludes: psychogenic:

- alopecia (316 and 704.0)
- dermatitis (316 and 692.-)
- eczema (316 and 691.9 or 692.-)
- urticaria (316 and 708.-)

306.4 *Gastrointestinal*

Aerophagy	Cyclical vomiting, psychogenic
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Excludes: cyclical vomiting NOS (536.2)
mucous colitis (316 and 564.1)
psychogenic:

- cardiospasm (316 and 530.0)
- duodenal ulcer (316 and 532.-)
- gastric ulcer (316 and 531.-)
- peptic ulcer (316 and 533.-)

306.5 *Genitourinary*

Psychogenic dysmenorrhoea

Excludes: dyspareunia (302.7)
enuresis (307.6)
frigidity (302.7)
impotence (302.7)

306.6 *Endocrine*306.7 *Organs of special sense*

Excludes: hysterical blindness or deafness (300.1)

306.8 *Other*

Teeth-grinding

306.9 *Unspecified*

Psychophysiologic disorder NOS	Psychosomatic disorder NOS
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307 Special symptoms or syndromes not elsewhere classified

Conditions in which an outstanding symptom or group of symptoms is not manifestly part of a more fundamental classifiable condition.

Excludes: when due to mental disorders classified elsewhere
when of organic origin

307.0 *Stammering and stuttering*

Disorders in the rhythm of speech, in which the individual knows precisely what he wishes to say, but at the time is unable to say it because of an involuntary, repetitive prolongation or cessation of a sound.

Excludes: dysphasia (784.5)
lispings or lalling (307.9)
retarded development of speech (315.3)

307.1 *Anorexia nervosa*

A disorder in which the main features are persistent active refusal to eat and marked loss of weight. The level of activity and alertness is characteristically high in relation to the degree of emaciation. Typically the disorder begins in teenage girls but it may sometimes begin before puberty and rarely it occurs in males. Amenorrhoea is usual and there may be a variety of other physiological changes including slow pulse and respiration, low body temperature and dependent oedema. Unusual eating habits and attitudes toward food are typical and sometimes starvation follows or alternates with periods of overeating. The accompanying psychiatric symptoms are diverse.

Excludes: eating disturbance NOS (307.5)
loss of appetite (783.0)
of nonorganic origin (307.5)

307.2 *Tics*

Disorders of no known organic origin in which the outstanding feature consists of quick, involuntary, apparently purposeless, and frequently repeated movements which are not due to any neurological condition. Any part of the body may be involved but the face is most frequently affected. Only one form of tic may be present, or there may be a combination of tics which are carried out simultaneously, alternatively or consecutively. Gilles de la Tourette's syndrome refers to a rare disorder occurring in individuals of any level of intelligence in which facial tics and tic-like throat noises become more marked and more generalized and in which later, whole words or short sentences [often with an obscene content] are ejaculated spasmodically and involuntarily. There is some overlap with other varieties of tic.

Excludes: nail-biting or thumb-sucking (307.9)
stereotypies occurring in isolation (307.3)
tics of organic origin (333.3)

307.3 *Stereotyped repetitive movements*

Disorders in which voluntary repetitive stereotyped movements, which are not due to any psychiatric or neurological condition, constitute the main feature. Includes headbanging, spasmus nutans, rocking, twirling, finger-flicking mannerisms and eye poking. Such move-

ments are particularly common in cases of mental retardation with sensory impairment or with environmental monotony.

Stereotypies NOS

Excludes: tics:

- NOS (307.2)
- of organic origin (333.3)

307.4 *Specific disorders of sleep*

This category should only be used when a more precise medical or psychiatric diagnosis cannot be made.

Hypersomnia	}	of nonorganic origin
Insomnia		
Inversion of sleep rhythm		
Nightmares		
Night terrors		
Sleepwalking		

Excludes: narcolepsy (347.0)
when of unspecified cause (780.5)

307.5 *Other and unspecified disorders of eating*

This category should only be used when a more precise medical or psychiatric diagnosis cannot be made.

Infantile feeding disturbances	}	of nonorganic origin
Loss of appetite		
Overeating		
Pica		
Psychogenic vomiting		

Excludes: anorexia:

- nervosa (307.1)
- of unspecified cause (783.0)
- overeating of unspecified cause (783.6)

vomiting:

- NOS (787.0)
- cyclical (536.2)
- psychogenic (306.4)

307.6 *Enuresis*

A disorder in which the main manifestation is a persistent involuntary voiding of urine by day or night which is considered abnormal for the age of the individual. Sometimes the child will have failed to gain bladder control and in other cases he will have gained control and then lost it. Episodic or fluctuating enuresis should be included. The disorder would not usually be diagnosed under the age of four years.

Enuresis (primary) (secondary) of nonorganic origin

Excludes: enuresis of unspecified cause (788.3)

307.7 *Encopresis*

A disorder in which the main manifestation is the persistent voluntary or involuntary passage of formed motions of normal or near-normal consistency into places not intended for that purpose in the individual's own sociocultural setting. Sometimes the child has failed to gain bowel control, and sometimes he has gained control but then later again became encopretic. There may be a variety of associated psychiatric symptoms and there may be smearing of faeces. The condition would not usually be diagnosed under the age of four years.

Encopresis (continuous) (discontinuous) of nonorganic origin

Excludes: encopresis of unspecified cause (787.6)

307.8 *Psychalgia*

Cases in which there are pains of mental origin, e.g., headache or backache, when a more precise medical or psychiatric diagnosis cannot be made.

Tension headache

Psychogenic backache

Excludes: migraine (346.-)

pains not specifically attributable to a psychological cause (in):

back (784.5)

headache (784.0)

joint (719.4)

limb (729.5)

lumbago (724.2)

rheumatic (729.0)

307.9 *Other and unspecified*

The use of this category should be discouraged. Most of the items listed in the inclusion terms are not indicative of psychiatric disorder and are included only because such terms may sometimes still appear as diagnoses.

Hair plucking

Masturbation

Lalling

Nail-biting

Lisping

Thumb-sucking

308 **Acute reaction to stress**

Very transient disorders of any severity and nature which occur in individuals without any apparent mental disorder in response to exceptional physical or mental stress, such as natural catastrophe or battle, and which usually subside within hours or days.

Catastrophic stress

Exhaustion delirium

Combat fatigue

Excludes: adjustment reaction (309.-)

308.0 *Predominant disturbance of emotions*

Panic states, excitability, fear, depressions and anxiety fulfilling the above criteria.

308.1 *Predominant disturbance of consciousness*

Fugues fulfilling the above criteria.

308.2 *Predominant psychomotor disturbance*

Agitation states, stupor fulfilling the above criteria.

308.3 *Other*

Acute situational disturbance

308.4 *Mixed*

Many gross stress reactions include several elements but whenever possible a specific coding under .0, .1, .2 or .3 should be made according to the *preponderant* type of disturbance. The category of mixed disorders should only be used when there is such an admixture that this cannot be done.

308.9 *Unspecified***309 *Adjustment reaction***

Mild or transient disorders lasting longer than acute stress reactions (308.-) which occur in individuals of any age without any apparent pre-existing mental disorder. Such disorders are often relatively circumscribed or situation-specific, are generally reversible, and usually last only a few months. They are usually closely related in time and content to stresses such as bereavement, migration or separation experiences. Reactions to major stress that last longer than a few days are also included here. In children such disorders are associated with no significant distortion of development.

Excludes: acute reaction to major stress (308.-)
neurotic disorders (300.-)

309.0 *Brief depressive reaction*

States of depression, not specifiable as manic-depressive, psychotic or neurotic, generally transient, in which the depressive symptoms are usually closely related in time and content to some stressful event.

Grief reaction

Excludes: affective psychoses (296.-)
neurotic depression (300.4)
prolonged depressive reaction (309.1)
psychogenic depressive psychosis (298.0)

309.1 *Prolonged depressive reaction*

States of depression, not specifiable as manic-depressive, psychotic or neurotic, generally long-lasting; usually developing in association with prolonged exposure to a stressful situation.

Excludes: affective psychoses (296.-)
brief depressive reaction (309.0)
neurotic depression (300.4)
psychogenic depressive psychosis (298.0)

309.2 *With predominant disturbance of other emotions*

States, fulfilling the general criteria for adjustment reaction, in which the main symptoms are emotional in type [anxiety, fear, worry, etc.] but not specifically depressive.

Abnormal separation anxiety Culture shock

309.3 *With predominant disturbance of conduct*

Mild or transient disorders, fulfilling the general criteria for adjustment reaction, in which the main disturbance predominantly involves a disturbance of conduct. For example, an adolescent grief reaction resulting in aggressive or antisocial disorder would be included here.

Excludes: disturbance of conduct NOS (312.-)
 dyssocial behaviour without manifest psychiatric disorder (V71.0)
 personality disorder with predominantly sociopathic or asocial manifestations (301.7)

309.4 *With mixed disturbance of emotions and conduct*

Disorders fulfilling the general definition in which both emotional disturbance and disturbance of conduct are prominent features.

309.8 *Other*

Adjustment reaction with elective mutism
 Hospitalism in children NOS

309.9 *Unspecified*

Adjustment reaction NOS Adaptation reaction NOS

310 **Specific nonpsychotic mental disorders following organic brain damage**

Note: This category should be used only for conditions where the *form* of the disorder is determined by the brain pathology.

Excludes: neuroses, personality disorders or other nonpsychotic conditions occurring in a form similar to that seen with functional disorders but in association with a physical condition; code to 300.-, 301.-, etc., and use additional code to identify the physical condition

310.0 *Frontal lobe syndrome*

Changes in behaviour following damage to the frontal areas of the brain or following interference with the connections of those areas. There is a general diminution of self-control, foresight, creativity and spontaneity, which may be manifest as increased irritability, selfishness, restlessness and lack of concern for others. Conscientiousness and powers of concentration are often diminished, but measurable deterioration of intellect or memory is not necessarily present. The overall picture is often one of emotional dullness, lack of drive and slowness; but, particularly in persons previously with energetic, restless or aggressive characteristics, there may be a change towards impulsiveness, boastfulness, temper

outbursts, silly fatuous humour, and the development of unrealistic ambitions; the direction of change usually depends upon the previous personality. A considerable degree of recovery is possible and may continue over the course of several years.

Lobotomy syndrome

Postleucotomy syndrome (state)

Excludes: postcontusional syndrome (310.2)

310.1 *Cognitive or personality change of other type*

Chronic, mild states of memory disturbance and intellectual deterioration, often accompanied by increased irritability, querulousness, lassitude and complaints of physical weakness. These states are often associated with old age, and may precede more severe states due to brain damage classifiable under dementia of any type (290.-, and 294.-) or any condition in 293.- (Transient organic psychotic conditions).

Mild memory disturbance

Organic psychosyndrome of nonpsychotic severity

310.2 *Postconcussional syndrome*

States occurring after generalized contusion of the brain, in which the symptom picture may resemble that of the frontal lobe syndrome (310.0) or that of any of the neurotic disorders (300.0-300.9), but in which in addition, headache, giddiness, fatigue, insomnia and a subjective feeling of impaired intellectual ability are usually prominent. Mood may fluctuate, and quite ordinary stress may produce exaggerated fear and apprehension. There may be marked intolerance of mental and physical exertion, undue sensitivity to noise, and hypochondriacal preoccupation. The symptoms are more common in persons who have previously suffered from neurotic or personality disorders, or when there is a possibility of compensation. This syndrome is particularly associated with the closed type of head injury when signs of localized brain damage are slight or absent, but it may also occur in other conditions.

Postcontusional syndrome (encephalopathy)

Status post commotio cerebri

Post-traumatic brain syndrome, nonpsychotic

Excludes: frontal lobe syndrome (310.0)

postencephalitic syndrome (310.8)

any organic psychotic conditions following head injury
(290.- to 294.0)

310.8 *Other*

Include here disorders resembling the postcontusional syndrome (310.2), associated with infective or other diseases of the brain or surrounding tissues.

Other focal (partial) organic psychosyndromes

310.9 *Unspecified*

311 **Depressive disorders, not elsewhere classified**

States of depression, usually of moderate but occasionally of marked intensity, which have no specifically manic-depressive or other psychotic depressive features and which do not

appear to be associated with stressful events or other features specified under neurotic depression.

Depressive disorder NOS

Depression NOS

Depressive state NOS

Excludes: acute reaction to major stress with depressive symptoms (308.0)
affective personality disorder (301.1)
affective psychoses (296.-)
brief depressive reaction (309.0)
disturbance of emotions specific to childhood and adolescence, with misery and unhappiness (313.1)
mixed adjustment reaction with depressive symptoms (309.4)
neurotic depression (300.4)
prolonged depressive adjustment reaction (309.1)
psychogenic depressive psychosis (298.0)

312 Disturbance of conduct not elsewhere classified

Disorders mainly involving aggressive and destructive behaviour and disorders involving delinquency. It should be used for abnormal behaviour, in individuals of any age, which gives rise to social disapproval but which is not part of any other psychiatric condition. Minor emotional disturbances may also be present. To be included, the behaviour—as judged by its frequency, severity and type of associations with other symptoms—must be abnormal in its context. Disturbances of conduct are distinguished from an adjustment reaction by a longer duration and by a lack of close relationship in time and content to some stress. They differ from a personality disorder by the absence of deeply ingrained maladaptive patterns of behaviour present from adolescence or earlier.

Excludes: adjustment reaction with disturbance of conduct (309.3)
drug dependence (304.-)
dyssocial behaviour without manifest psychiatric disorder (V71.0)
personality disorder with predominantly sociopathic or asocial manifestations (301.7)
sexual deviations (302.-)

312.0 *Unsocialized disturbance of conduct*

Disorders characterized by behaviours such as defiance, disobedience, quarrelsomeness, aggression, destructive behaviour, tantrums, solitary stealing, lying, teasing, bullying and disturbed relationships with others. The defiance may sometimes take the form of sexual misconduct.

Unsocialized aggressive disorder

312.1 *Socialized disturbance of conduct*

Disorders in individuals who have acquired the values or behaviours of a delinquent peer group to whom they are loyal and with whom they characteristically steal, play truant, and stay out late at night. There may also be promiscuity.

Group delinquency

Excludes: gang activity without manifest psychiatric disorder (V71.0)

312.2 *Compulsive conduct disorder*

Disorder of conduct or delinquent act which is specifically compulsive in origin.

Kleptomania

312.3 *Mixed disturbance of conduct and emotions*

Disorders involving behaviours listed for 312.0 and 312.1 but in which there is also *considerable* emotional disturbance as shown for example by anxiety, misery or obsessive manifestations.

Neurotic delinquency

Excludes: compulsive conduct disorder (312.2)

312.8 *Other*

312.9 *Unspecified*

313 **Disturbance of emotions specific to childhood and adolescence**

Less well differentiated emotional disorders characteristic of the childhood period. Where the emotional disorder takes the form of a neurotic disorder described under 300.-, the appropriate 300.- coding should be made. This category differs from category 308.- in terms of longer duration and by the lack of close relationship in time and content to some stress.

Excludes: adjustment reaction (309.-)
masturbation, nail-biting, thumb-sucking and other isolated symptoms (307.-)

313.0 *With anxiety and fearfulness*

Ill-defined emotional disorders characteristic of childhood in which the main symptoms involve anxiety and fearfulness. Many cases of school refusal or elective mutism might be included here.

Overanxious reaction of childhood or adolescence

Excludes: abnormal separation anxiety (309.2)
anxiety states (300.0)
hospitalism in children (309.8)
phobic state (300.2)

313.1 *With misery and unhappiness*

Emotional disorders characteristic of childhood in which the main symptoms involve misery and unhappiness. There may also be eating and sleep disturbances.

Excludes: depressive neurosis (300.4)

313.2 *With sensitivity, shyness and social withdrawal*

Emotional disorders characteristic of childhood in which the main symptoms involve sensitivity, shyness, or social withdrawal. Some cases of elective mutism might be included here.

Withdrawing reaction of childhood or adolescence

Excludes: infantile autism (299.0)
schizoid personality (301.2)
schizophrenia (295.-)

313.3 *Relationship problems*

Emotional disorders characteristic of childhood in which the main symptoms involve relationship problems.

Sibling jealousy

Excludes: relationship problems associated with aggression, destruction or other forms of conduct disturbance (312.-)

313.8 *Other or mixed*

Many emotional disorders of childhood include several elements but whenever possible a specific coding under .0, .1, .2 or .3 should be made according to the *preponderant* type of disturbance. The category of mixed disorders should only be used when there is such an admixture that this cannot be done.

313.9 *Unspecified*

314 **Hyperkinetic syndrome of childhood**

Disorders in which the essential features are short attention span and distractibility. In early childhood the most striking symptom is disinhibited, poorly organized and poorly regulated extreme overactivity but in adolescence this may be replaced by underactivity. Impulsiveness, marked mood fluctuations and aggression are also common symptoms. Delays in the development of specific skills are often present and disturbed, poor relationships are common. If the hyperkinesis is symptomatic of an underlying disorder, code the underlying disorder instead.

314.0 *Simple disturbance of activity and attention*

Cases in which short attention span, distractibility, and overactivity are the main manifestations without significant disturbance of conduct or delay in specific skills.

Overactivity NOS

314.1 *Hyperkinesis with developmental delay*

Cases in which the hyperkinetic syndrome is associated with speech delay, clumsiness, reading difficulties or other delays in specific skills.

Developmental disorder of hyperkinesis

Use additional code to identify any associated neurological disorder

314.2 *Hyperkinetic conduct disorder*

Cases in which the hyperkinetic syndrome is associated with marked conduct disturbance but not developmental delay.

Hyperkinetic conduct disorder

Excludes: hyperkinesis with significant delays in specific skills (314.1)

314.8 *Other*314.9 *Unspecified*

Hyperkinetic reaction of childhood Hyperkinetic syndrome NOS
or adolescence NOS

315 Specific delays in development

A group of disorders in which a specific delay in development is the main feature. In each case development is related to biological maturation but it is also influenced by nonbiological factors and the coding carries no aetiological implications.

Excludes: when due to a neurological disorder (320-389)

315.0 *Specific reading retardation*

Disorders in which the main feature is a serious impairment in the development of reading or spelling skills which is not explicable in terms of general intellectual retardation or of inadequate schooling. Speech or language difficulties, impaired right-left differentiation, perceptuo-motor problems, and coding difficulties are frequently associated. Similar problems are often present in other members of the family. Adverse psychosocial factors may be present.

Developmental dyslexia

Specific spelling difficulty

315.1 *Specific arithmetical retardation*

Disorders in which the main feature is a serious impairment in the development of arithmetical skills which is not explicable in terms of general intellectual retardation or of inadequate schooling.

Dyscalculia

315.2 *Other specific learning difficulties*

Disorders in which the main feature is a serious impairment in the development of other learning skills which are not explicable in terms of general intellectual retardation or of inadequate schooling.

Excludes: specific arithmetical retardation (315.1)
 specific reading retardation (315.0)

315.3 *Developmental speech or language disorder*

Disorders in which the main feature is a serious impairment in the development of speech or language [syntax or semantics] which is not explicable in terms of general intellectual retardation. Most commonly there is a delay in the development of normal word-sound production resulting in defects of articulation. Omissions or substitutions of consonants are most frequent. There may also be a delay in the production of spoken language. Rarely, there is also a developmental delay in the comprehension of sounds. Includes cases in which delay is largely due to environmental privation.

Developmental aphasia

Dyslalia

Excludes: acquired aphasia (784.3)
 elective mutism (309.8, 313.0 or 313.2)
 lisp and lalling (307.9)
 stammering and stuttering (307.0)

315.4 *Specific motor retardation*

Disorders in which the main feature is a serious impairment in the development of motor coordination which is not explicable in terms of general intellectual retardation. The clumsiness is commonly associated with perceptual difficulties.

Clumsiness syndrome

Dyspraxia syndrome

315.5 *Mixed development disorder*

A delay in the development of one specific skill [e.g., reading, arithmetic, speech or coordination] is frequently associated with lesser delays in other skills. When this occurs the coding should be made according to the skill most seriously impaired. The mixed category should be used only where the mixture of delayed skills is such that no one skill is preponderantly affected.

315.8 *Other*

315.9 *Unspecified*

Developmental disorder NOS

316 **Psychic factors associated with diseases classified elsewhere**

Mental disturbances or psychic factors of any type thought to have played a major part in the aetiology of physical conditions, usually involving tissue damage, classified elsewhere. The mental disturbance is usually mild and nonspecific and psychic factors [worry, fear, conflict, etc.] may be present without any overt psychiatric disorder. Use an additional code to identify the physical condition. In the rare instance that an overt psychiatric disorder is thought to have caused a physical condition, use a second additional code to record the psychiatric diagnosis.

Examples of the use of this category are:

psychogenic:

asthma 316 and 493.9

dermatitis 316 and 692.-

eczema 316 and 691.- or 692.-

gastric ulcer 316 and 531.-

mucous colitis 316 and 564.1

ulcerative colitis 316 and 556

urticaria 316 and 708.-

psychosocial dwarfism 316 and 259.4

Excludes: physical symptoms and physiological malfunctions, not involving tissue damage, of mental origin (306.-)

MENTAL RETARDATION (317-319)

A condition of arrested or incomplete development of mind which is especially characterized by subnormality of intelligence. The coding should be made on the individual's *current* level of functioning *without regard to its nature* or causation—such as psychosis, cultural deprivation, Down's syndrome etc. Where there is a specific cognitive handicap—such as in speech—the four-digit coding should be based on assessments of cognition *outside the area*

of *specific handicap*. The assessment of intellectual level should be based on whatever information is available, including clinical evidence, adaptive behaviour and psychometric findings. The IQ levels given are based on a test with a mean of 100 and a standard deviation of 15—such as the Wechsler scales. They are provided only as a guide and should not be applied rigidly. Mental retardation often involves psychiatric disturbances and may often develop as a result of some physical disease or injury. In these cases, an additional code or codes should be used to identify any associated condition, psychiatric or physical. The Impairment and Handicap codes should also be consulted.

317 Mild mental retardation

Feeble-minded	Moron
High-grade defect	IQ 50-70
Mild mental subnormality	

318 Other specific mental retardation

318.0 *Moderate mental retardation*

Imbecile	Moderate mental subnormality
IQ 35-49	

318.1 *Severe mental retardation*

IQ 20-34	Severe mental subnormality
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318.2 *Profound mental retardation*

Idiocy	Profound mental subnormality
IQ under 20	

319 Unspecified mental retardation

Mental deficiency NOS	Mental subnormality NOS
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ICD-9-CM CLASSIFICATION OF MENTAL DISORDERS

(without inclusion and exclusion terms)

From *The International Classification of Diseases, Clinical Modification, Ninth Revision (ICD-9-CM)*, Commission on Professional and Hospital Activities, Edwards Bros., Ann Arbor, MI, 1979.

Italics indicate specific ICD-9-CM codes and their categories not included in DSM-III. The lozenge symbol (◊) printed in the left margin preceding the disease code denotes a four-digit rubric unique to ICD-9-CM. The contents of these rubrics in ICD-9-CM are not the same as those in ICD-9.

MENTAL DISORDERS (290-319)

PSYCHOSES (290-299)

ORGANIC PSYCHOTIC CONDITIONS (290-294)

290 Senile and presenile organic psychotic conditions

290.0 Senile dementia, uncomplicated

290.1 Presenile dementia

290.10 Presenile dementia, uncomplicated

290.11 Presenile dementia with delirium

290.12 Presenile dementia with delusional features

290.13 Presenile dementia with depressive features

290.2 Senile dementia with delusional or depressive features

290.20 Senile dementia with delusional features

290.21 Senile dementia with depressive features

290.3 Senile dementia with delirium

290.4 Arteriosclerotic dementia

290.40 Arteriosclerotic dementia, uncomplicated

290.41 Arteriosclerotic dementia with delirium

290.42 Arteriosclerotic dementia with delusional features

290.43 Arteriosclerotic dementia with depressive features

290.8 Other specified senile psychotic conditions

290.9 Unspecified senile psychotic condition

291 Alcoholic psychoses

291.0 Alcohol withdrawal delirium

291.1 Alcohol amnestic syndrome

291.2 Other alcoholic dementia

291.3 Alcohol withdrawal hallucinosis

291.4 Idiosyncratic alcohol intoxication

291.5 Alcoholic jealousy

291.8 Other specified alcoholic psychosis

291.9 Unspecified alcoholic psychosis

292 Drug psychoses

292.0 Drug withdrawal syndrome

292.1 Paranoid and/or hallucinatory states induced by drugs

292.11 Drug-induced organic delusional syndrome

292.12 Drug-induced hallucinosis

292.2 *Pathological drug intoxication*

- 292.8 Other specified drug-induced mental disorders
 - 292.81 Drug-induced delirium
 - 292.82 Drug-induced dementia
 - 292.83 Drug-induced amnestic syndrome
 - 292.84 Drug-induced organic affective syndrome
 - 292.89 Other
- 292.9 Unspecified drug-induced mental disorder

293 Transient organic psychotic conditions

- 293.0 Acute delirium
- 293.1 *Subacute delirium*
- 293.8 Other specified transient organic mental disorders
 - 293.81 Organic delusional syndrome
 - 293.82 Organic hallucinosis syndrome
 - 293.83 Organic affective syndrome
 - 293.89 Other
- 293.9 Unspecified transient organic mental disorder

294 Other organic psychotic conditions (chronic)

- 294.0 Amnestic syndrome
- 294.1 Dementia in conditions classified elsewhere
- 294.8 Other specified organic brain syndromes (chronic)
- 294.9 Unspecified organic brain syndrome (chronic)

OTHER PSYCHOSES (295-299)

295 Schizophrenic disorders

- 295.0 *Simple type*
- 295.1 Disorganized type
- 295.2 Catatonic type
- 295.3 Paranoid type
- 295.4 Acute schizophrenic episode
- 295.5 *Latent schizophrenia*
- 295.6 Residual schizophrenia
- 295.7 Schizo-affective type
- 295.8 Other specified types of schizophrenia
- 295.9 Unspecified schizophrenia

296 Affective psychoses

- 296.0 *Manic disorder, single episode*
- 296.1 *Manic disorder, recurrent episode*
- 296.2 Major depressive disorder, single episode
- 296.3 Major depressive disorder, recurrent episode
- 296.4 Bipolar affective disorder, manic
- 296.5 Bipolar affective disorder, depressed
- 296.6 Bipolar affective disorder, mixed
- 296.7 Bipolar affective disorder, unspecified
- 296.8 Manic-depressive psychosis, other and unspecified
 - 296.80 Manic-depressive psychosis, unspecified
 - 296.81 Atypical manic disorder
 - 296.82 Atypical depressive disorder
 - 296.89 Other
- 296.9 Other and unspecified affective psychoses
 - 296.90 Unspecified affective psychosis
 - 296.99 Other specified affective psychoses

297 Paranoid states

- 297.0 *Paranoid state, simple*
- 297.1 Paranoia
- 297.2 *Paraphrenia*
- 297.3 Shared paranoid disorder
- 297.8 Other specified paranoid states
- 297.9 Unspecified paranoid state

298 Other nonorganic psychoses

- 298.0 *Depressive type psychosis*
- 298.1 *Excitative type psychosis*
- 298.2 *Reactive confusion*
- 298.3 Acute paranoid reaction
- 298.4 *Psychogenic paranoid psychosis*
- 298.8 Other and unspecified reactive psychosis
- 298.9 Unspecified psychosis

299 Psychoses with origin specific to childhood

- 299.0 Infantile autism

- 299.1 *Disintegrative psychosis*
- 299.8 Other specified early childhood psychoses
- 299.9 Unspecified

NEUROTIC DISORDERS, PERSONALITY DISORDERS, AND OTHER NONPSYCHOTIC MENTAL DISORDERS (300-316)

300 Neurotic disorders

300.0 Anxiety states

- 300.00 Anxiety state, unspecified
- 300.01 Panic disorder
- 300.02 Generalized anxiety disorder
- 300.09 Other

300.1 Hysteria

- 300.10 Hysteria, unspecified
- 300.11 Conversion disorder
- 300.12 Psychogenic amnesia
- 300.13 Psychogenic fugue
- 300.14 Multiple personality
- 300.15 Dissociative disorder or reaction, unspecified
- 300.16 Factitious illness with psychological symptoms
- 300.19 Other and unspecified factitious illness

300.2 Phobic disorders

- 300.20 Phobia, unspecified
- 300.21 Agoraphobia with panic attacks
- 300.22 Agoraphobia without mention of panic attacks
- 300.23 Social phobia
- 300.29 Other isolated or simple phobias

300.3 Obsessive-compulsive disorders

300.4 Neurotic depression

300.5 *Neurasthenia*

300.6 Depersonalization syndrome

300.7 Hypochondriasis

300.8 Other neurotic disorders

- 300.81 Somatization disorder
- 300.89 Other

300.9 Unspecified neurotic disorder

301 Personality disorders

301.0 Paranoid personality disorder

301.1 *Affective personality disorder*

301.10 Affective personality disorder, unspecified

301.11 *Chronic hypomanic personality disorder*

301.12 *Chronic depressive personality disorder*

301.13 Cyclothymic disorder

301.2 Schizoid personality disorder

301.20 Schizoid personality disorder, unspecified

301.21 Introverted personality

301.22 Schizotypal personality

301.3 *Explosive personality disorder*

301.4 Compulsive personality disorder

301.5 Histrionic personality disorder

301.50 Histrionic personality disorder, unspecified

301.51 Chronic factitious illness with physical symptoms

301.59 Other histrionic personality disorder

301.6 Dependent personality disorder

301.7 Antisocial personality disorder

301.8 Other personality disorders

301.81 Narcissistic personality

301.82 Avoidant personality

301.83 Borderline personality

301.84 Passive-aggressive personality

301.89 Other

301.9 Unspecified personality disorder

302 Sexual deviations and disorders

302.0 Homosexuality

302.1 Zoophilia

302.2 Pedophilia

302.3 Transvestism

302.4 Exhibitionism

302.5 Trans-sexualism

302.6 Disorders of psychosexual identity

302.7 Psychosexual dysfunction

302.70 Psychosexual dysfunction, unspecified

302.71 With inhibited sexual desire

302.72 With inhibited sexual excitement

302.73 With inhibited female orgasm

302.74 With inhibited male orgasm

302.75 With premature ejaculation

302.76 With functional dyspareunia

302.79 With other specified psychosexual dysfunctions

302.8 Other specified psychosexual disorders

302.81 Fetishism

302.82 Voyeurism

302.83 Sexual masochism

302.84 Sexual sadism

302.85 Gender identity disorder of adolescent or adult life

302.89 Other

302.9 Unspecified psychosexual disorder

303 Alcohol dependence syndrome

303.0 Acute alcoholic intoxication

303.9 Other and unspecified alcohol dependence

304 Drug dependence

304.0 Opioid type dependence

304.1 Barbiturate and similarly acting sedative or hypnotic dependence

304.2 *Cocaine dependence*

304.3 Cannabis dependence

304.4 Amphetamine and other psychostimulant dependence

304.5 *Hallucinogen dependence*

304.6 Other specified drug dependence

304.7 Combinations of opioid type drug with any other

304.8 Combinations of drug dependence excluding opioid type drug

304.9 Unspecified drug dependence

305 Nondependent abuse of drugs

305.0 Alcohol abuse

305.1 Tobacco use disorder

305.2 Cannabis abuse

305.3 Hallucinogen abuse

305.4 Barbiturate and similarly acting sedative or hypnotic abuse

305.5 Opioid abuse

305.6 Cocaine abuse

305.7 Amphetamine or related acting sympathomimetic abuse

305.8 *Antidepressant type abuse*

305.9 Other, mixed, or unspecified drug abuse

306 *Physiological malfunction arising from mental factors*

306.0 *Musculoskeletal*

306.1 *Respiratory*

306.2 *Cardiovascular*

306.3 *Skin*

306.4 *Gastrointestinal*

306.5 *Genitourinary*

306.50 Psychogenic genitourinary malfunction, unspecified

306.51 Psychogenic vaginismus

306.52 *Psychogenic dysmenorrhea*

306.53 *Psychogenic dysuria*

306.59 Other

306.6 *Endocrine*

306.7 *Organs of special sense*

306.8 Other specified psychophysiological malfunction

306.9 Unspecified psychophysiological malfunction

307 Special symptoms or syndromes, not elsewhere classified

307.0 Stammering and stuttering

307.1 Anorexia nervosa

307.2 Tics

307.20 Tic disorder, unspecified

307.21 Transient tic disorder of childhood

307.22 Chronic motor tic disorder

307.23 Gilles de la Tourette's disorder

307.3 Stereotyped repetitive movements

307.4 Specific disorders of sleep of nonorganic origin

307.40 *Nonorganic sleep disorder, unspecified*307.41 *Transient disorder of initiating or maintaining sleep*307.42 *Persistent disorder of initiating or maintaining sleep*307.43 *Transient disorder of initiating or maintaining wakefulness*307.44 *Persistent disorder of initiating or maintaining wakefulness*307.45 *Phase-shift disruption of 24-hour sleep-wake cycle*

307.46 Somnambulism or night terrors

307.47 Other dysfunctions of sleep stages or arousal from sleep

307.48 *Repetitive intrusions of sleep*

307.49 Other

307.5 Other and unspecified disorders of eating

307.50 Eating disorder, unspecified

307.51 Bulimia

307.52 Pica

307.53 Psychogenic rumination

307.54 *Psychogenic vomiting*

307.59 Other

307.6 Enuresis

307.7 Encopresis

307.8 Psychalgia

307.80 Psychogenic pain, site unspecified

307.81 *Tension headache*

307.89 Other

307.9 Other and unspecified special symptoms or syndromes, not elsewhere classified

308 Acute reaction to stress

- 308.0 *Predominant disturbance of emotions*
- 308.1 *Predominant disturbance of consciousness*
- 308.2 *Predominant psychomotor disturbance*
- 308.3 Other acute reactions to stress
- 308.4 Mixed disorders as reaction to stress
- 308.9 Unspecified acute reaction to stress

309 Adjustment reaction

- 309.0 Brief depressive reaction
- 309.1 *Prolonged depressive reaction*
- 309.2 With predominant disturbance of other emotions
 - 309.21 Separation anxiety disorder
 - 309.22 *Emancipation disorder of adolescence and early adult life*
 - 309.23 Specific academic or work inhibition
 - 309.24 Adjustment reaction with anxious mood
 - 309.28 Adjustment reaction with mixed emotional features
 - 309.29 Other
- 309.3 With predominant disturbance of conduct
- 309.4 With mixed disturbance of emotions and conduct
- 309.8 Other specified adjustment reactions
 - 309.81 Prolonged posttraumatic stress disorder
 - 309.82 Adjustment reaction with physical symptoms
 - 309.83 Adjustment reaction with withdrawal
 - 309.89 Other
- 309.9 Unspecified adjustment reaction

310 Specific nonpsychotic mental disorders due to organic brain damage

- 310.0 *Frontal lobe syndrome*
- 310.1 Organic personality syndrome
- 310.2 *Postconcussion syndrome*
- 310.8 Other specified nonpsychotic mental disorders following organic brain damage

310.9 Unspecified nonpsychotic mental disorder following organic brain damage

311 *Depressive disorder, not elsewhere classified*

312 Disturbance of conduct, not elsewhere classified

□312.0 Undersocialized conduct disorder, aggressive type

□312.1 Undersocialized conduct disorder, unaggressive type

□312.2 Socialized conduct disorder

□312.3 Disorders of impulse control, not elsewhere classified

312.30 Impulse control disorder, unspecified

312.31 Pathological gambling

312.32 Kleptomania

312.33 Pyromania

312.34 Intermittent explosive disorder

312.35 Isolated explosive disorder

312.39 Other

□312.4 *Mixed disturbance of conduct and emotions*

312.8 Other specified disturbances of conduct, not elsewhere classified

312.9 Unspecified disturbance of conduct

313 Disturbance of emotions specific to childhood and adolescence

313.0 Overanxious disorder

313.1 *Misery and unhappiness disorder*

313.2 Sensitivity, shyness, and social withdrawal disorder

313.21 Shyness disorder of childhood

313.22 Introverted disorder of childhood

313.23 Elective mutism

313.3 *Relationship problems*

313.8 Other or mixed emotional disturbances of childhood or adolescence

313.81 Oppositional disorder

313.82 Identity disorder

313.83 *Academic underachievement disorder*

313.89 Other

313.9 Unspecified emotional disturbance of childhood or adolescence

- 314 Hyperkinetic syndrome of childhood
 - 314.0 Attention deficit disorder
 - 314.00 Without mention of hyperactivity
 - 314.01 With hyperactivity
 - 314.1 *Hyperkinesis with developmental delay*
 - 314.2 *Hyperkinetic conduct disorder*
 - 314.8 Other specified manifestations of hyperkinetic syndrome
 - 314.9 Unspecified hyperkinetic syndrome
- 315 Specific delays in development
 - 315.0 Specific reading disorder
 - 315.00 Reading disorder, unspecified
 - 315.01 *Alexia*
 - 315.02 *Developmental dyslexia*
 - 315.09 Other
 - 315.1 Specific arithmetical disorder
 - 315.2 Other specific learning difficulties
 - 315.3 Developmental speech or language disorder
 - 315.31 Developmental language disorder
 - 315.39 Other
 - 315.4 *Coordination disorder*
 - 315.5 Mixed development disorder
 - 315.8 Other specified delays in development
 - 315.9 Unspecified delay in development
- 316 Psychic factors associated with diseases classified elsewhere

MENTAL RETARDATION (317-319)

- 317 Mild mental retardation
- 318 Other specified mental retardation
 - 318.0 Moderate mental retardation
 - 318.1 Severe mental retardation
 - 318.2 Profound mental retardation
- 319 Unspecified mental retardation

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**The Diagnostic
Classification of Sleep
and Arousal Disorders***

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Appendix E:

The Diagnostic Classification of Sleep and Arousal Disorders*

of the
Association of Sleep Disorders Centers
and the
Association for the Psychophysiological Study of Sleep

A. DIMS: DISORDERS OF INITIATING AND MAINTAINING SLEEP (Insomnias)

1. Psychophysiological

- a. Transient and Situational
- b. Persistent

2. *associated with*

Psychiatric Disorders

- a. Symptom and Personality Disorders
- b. Affective Disorders
- c. Other Functional Psychoses

3. *associated with*

Use of Drugs and Alcohol

- a. Tolerance to or Withdrawal from CNS Depressants
- b. Sustained Use of CNS Stimulants
- c. Sustained Use of or Withdrawal from Other Drugs
- d. Chronic Alcoholism

4. *associated with*

Sleep-Induced Respiratory Impairment

- a. Sleep Apnea DIMS Syndrome
- b. Alveolar Hypoventilation DIMS Syndrome

* Prepared by the Sleep Disorders Classification Committee of the Association of Sleep Disorders Centers: Howard P. Roffwarg, M.D. (Chair), Robert W. Clark, M.D., Christian Guilleminault, M.D., Peter J. Hauri, Ph.D., David J. Kupfer, M.D., Laughton E. Miles, M.D., Ph.D., Helmut S. Schmidt, M.D., Vincent P. Zarcone, Jr., M.D., and Frank J. Zorick, M.D. with William C. Dement, M.D., Ph.D. and Allan Rechtschaffen, Ph.D. as consultants. This is an outline of the first diagnostic classification system endorsed by both national sleep investigator societies. It represents a major consensus in the sleep disorders field. As published in *Sleep*, Volume 2, Number 1, 1979, published by Raven Press, the classification system contains a detailed description of each condition, differential diagnosis, background material, references, and a glossary.

5. *associated with*

Sleep-related (Nocturnal) Myoclonus and "Restless Legs" Syndromes

- a. Sleep-related (Nocturnal) Myoclonus
- b. "Restless Legs"

6. *associated with*

Other Medical, Toxic, and Environmental Conditions

7. **Childhood Onset DIMS**

8. *associated with*

Other DIMS Conditions

- a. Repeated REMS Interruptions
- b. Atypical Polysomnographic Features
- c. Not Otherwise Specified

9. **No DIMS Abnormality**

- a. Short Sleeper
- b. Subjective DIMS Complaints without Objective Findings
- c. Not Otherwise Specified

* * *

B. DOES: DISORDERS OF EXCESSIVE SOMNOLENCE

1. **Psychophysiological**

- a. Transient and Situational
- b. Persistent

2. *associated with*

Psychiatric Disorders

- a. Affective Disorders
- b. Other Disorders

3. *associated with*

Use of Drugs and Alcohol

- a. Tolerance to or Withdrawal from CNS Stimulants
- b. Sustained Use of CNS Depressants

4. **Sleep-Induced Respiratory Impairment**

- a. Sleep Apnea DOES Syndrome
- b. Alveolar Hypoventilation DOES Syndrome

5. *associated with*

Sleep-related (Nocturnal) Myoclonus and "Restless Legs" Syndromes

- a. Sleep-related (Nocturnal) Myoclonus
- b. "Restless Legs"

6. **Narcolepsy**

7. **Idiopathic CNS Hypersomnolence**

8. *associated with*

Other Medical, Toxic, and Environmental Conditions

9. **Other DOES Conditions**

- a. Intermittent DOES (Periodic) Syndromes
 - i. Kleine-Levin Syndrome
 - ii. Menstrual Associated Syndrome
- b. Insufficient Sleep
- c. Disorder of Initiating Wakefulness (Sleep Drunkenness)
- d. Not Otherwise Specified

10. **No DOES Abnormality**

- a. Long Sleeper
- b. Subjective DOES Complaint without Objective Findings
- c. Not Otherwise Specified

* * *

C. DISORDERS OF THE SLEEP-WAKE SCHEDULE

1. **Transient**

- a. Rapid Time Zone Change ("Jet Lag") Syndrome
- b. "Workshift" Change in Conventional Sleep-Wake Schedule

2. **Persistent**

- a. Frequently Changing Sleep-Wake Schedule
- b. Delayed Sleep Phase Syndrome
- c. Advanced Sleep Phase Syndrome
- d. Non-24-Hour Sleep-Wake Schedule
- e. Irregular Sleep-Wake Pattern
- f. Not Otherwise Specified

* * *

D. DYSFUNCTIONS ASSOCIATED WITH SLEEP, SLEEP STAGES, OR PARTIAL AROUSALS (PARASOMNIAS)

- 1. Sleepwalking (Somnambulism)**
- 2. Sleep Terror (Pavor Nocturnus, Incubus)**
- 3. Sleep-related Enuresis**
- 4. Other Dysfunctions**
 - a. Dream Anxiety Attacks (Nightmares)
 - b. Sleep-related Epileptic Seizures
 - c. Sleep-related Bruxism
 - d. Sleep-related Headbanging (Jactatio Capitus Nocturnus)
 - e. Familial Sleep Paralysis
 - f. Impaired Penile Tumescence during Sleep
 - g. Sleep-related Painful Erections
 - h. Sleep-related Cluster Headaches and Chronic Paroxysmal Hemicrania
 - i. Sleep-related Abnormal Swallowing Syndrome
 - j. Sleep-related Asthma
 - k. Sleep-related Cardiovascular Symptoms
 - l. Sleep-related Gastro-Esophageal Reflux
 - m. Sleep-related Hemolysis (Paroxysmal Nocturnal Hemoglobinuria)
 - n. Asymptomatic Polysomnographic Finding (specify condition)
 - o. Not Otherwise Specified